

Part 3

Part 3, you will hear two university students, discussing on Obesity and Health. First you have some time to look at questions 21 to 25.

Now listen carefully and answer questions 21 to 25.

Nancy: Hey Mark, have you had a chance to go through all the research materials for our health seminar next week?

Mark: Yeah, I just finished them last night. There were some really surprising findings, especially about how we measure obesity. What did you think about the BMI discussion?

Nancy: Oh my gosh, don't even get me started on BMI! (21) It's just out-of-date. I was shocked to learn that doctors still rely on it. The whole concept is fundamentally flawed because it treats muscle and fat as the same thing.

Mark: That does seem problematic when you put it that way. I always thought BMI was at least a good starting point for identifying potential weight issues.

Nancy: The problem isn't that it fails to define obesity *precisely* - it actually does provide a definition. And the data itself is reliable - the issue is with the calculation method. We desperately need a new system that accounts for body composition.

Mark: I see what you mean. Speaking of weight issues, what was your take on that popular weight-loss television show they analyzed?

Nancy: The one featuring all those fitness experts? Honestly, the main problem was their selection of experts. (22) They only featured people who were already at their ideal weight - some were too young and thin! So, the audience simply can't connect to them at all.

Mark: Yes, I agree. The experts themselves are highly experienced, but the show still falls short in this regard. They should feature people with average or even overweight body types - that would be far more meaningful.

Nancy: Exactly! So, in your opinion, what should be the main focus for these programs?

Mark: Well, I think the methods don't have to work for everybody - just being relatable and giving people some practical ideas is what's more important.

Nancy: Yeah, I agree. (23) And the concentration should be on boosting consciousness about overall fitness and health for everyone, not just targeting certain group of people. The current approach creates this stigma that only a small number of people need to worry about their health.

Mark: So, do you know what worries them even more than obesity itself?

Nancy: While obesity itself leads to higher living expenses and greater health risks, but these aren't their primary concerns. Let me check what other aspects we haven't covered yet - (24) the psychological dimension, like social impacts. For instance, the judgmental stares and ridicule people face, or how some might experience severe isolation.

Mark: Yeah, beyond the obesity itself, those can be just as damaging.

Nancy: Absolutely. On a more positive note, I was encouraged by what we read about community exercise programs. Did you see how many options are available now?

Mark: Yes! (25) Between public parks with fitness equipment, community centers offering free classes, and all the walking trails, people can easily access to those facilities.

Nancy: Yes, and what's more, it's completely safe and doesn't require any additional doctor's supervision.

Before you hear the rest of the talk, you have some time to look at questions 26 to 30.

Now listen and answer questions 26 to 30.

Mark: The expert opinions section was really eye-opening. Dr. Carter's perspective was interesting - (26) She has observed that many food producers are slowly cutting down on the amount of sugar in their offerings. This gradual reformulation appears to be an industry-wide response to growing health concerns.

Nancy: Yes, but Prof. Mitchell's take was more nuanced. (27) He emphasized that although food corporations sincerely aim to develop more nutritious products, significant sugar reduction risks substantially damaging profits by diminishing product appeal, forcing them to find a middle ground.

Mark: And Ms. Langley's position was quite different though - (28) She believes government rules and laws limit people's choices. These restrictions are meant to protect public health, but some argue they go too far in controlling personal freedom.

Nancy: (29) Which contrasts sharply with Mr. Harris's view that governments must avoid implementing sugar taxes without comprehensive planning. He argues that such impulsive fiscal measures often fail to account for some people, potentially creating unintended consequences for lower-income populations.

Mark: And then there was Dr. Reynolds, pointing out that - (30) While genetics do play a role in weight, parents still need to monitor their children's sugar intake. Even if a child is predisposed to obesity, excessive sugar can worsen health risks, so parental guidance remains crucial.

Nancy: Did any of the experts mention that exercise alone isn't enough to combat obesity?

Mark: Actually no, that point wasn't attributed to any specific expert. I wonder if that will come up in our seminar discussion.

Nancy: Probably. There's still so much to consider about this complex issue. Maybe we should...

That's the end of Part 3. You now have half a minute to check your answers.

Part 3

Part 3, you will hear two university students, Peter and Emma, discussing their research projects. First you have some time to look at questions 21 to 25.

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Peter: Hey Emma! Finally submitted my social science field report. Three months tracking community recycling habits – exhausted but relieved!

Emma: Congrats! It might have been time-consuming, but ultimately very meaningful, I think. So, what would you say was *the biggest significance* of running such activities as a social scientist?

Peter:

Well honestly? In terms of theoretical knowledge, there's so much to learn, and it truly helps improve our academic abilities. Additionally, active participation in community activities has effectively developed my practical skills. (21) But the most important thing is to gather reliable real-world data. Our sensors recorded actual bin usage – that evidence is priceless. Surveys alone can't capture behavioral patterns.

Emma: Makes sense! I knew this project would involve some advanced technological equipment, which I was really curious about. (22) But ultimately, I decided to join because the location was close to home - offering greater convenience!

Peter: Yeah, anyway, it's definitely worth participating in!

Emma: Absolutely! And I'm thinking about the research method, what methodology did you use, Peter?

Peter: Well, I've actually spent a long time considering this, we ruled out **long-term tracking** first – too many variables in real-world settings. (23) Instead, we created a standardized laboratory test, with fixed temperature and lighting. We planned to try some short-term surveys as well, but it didn't work, so we gave up this methodology.

Emma: Smart! But keeping volunteers was tough. Ours quit halfway last term. Probably because of all the repetition - people just found it really boring. Maybe changing up the weekly tasks would keep people from quitting.

Peter: Alternatively, implementing a bonus payment system may also work well. (24) However, I think publicity may be the best way for participant retention.

Emma: Yeah, I agree.

Peter: So, what are the next steps required for your statistics project?

Emma: Well, I currently have multiple pending tasks, including uncompleted data analysis and volunteer interviews assigned by my tutor. (25) However, I think prioritizing the creation of a chart to organize existing data would be my following step.

Peter: Sure thing!

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Peter: What's your current project? Still the astronomy club?

Emma: Yeah! Coordinating four citizen-science programs. Each has unique headaches. Take **Meteor Tracker** for example – teams install cameras in rural areas. (26) We need authorization from landowners. We can not carry out the program unless they allow us to do so.

Peter: And what about the **Snow Sampling** project? Sounded cool.

Emma: Collecting cosmic dust! But filters must cover the whole town. (27) We need to promote active student involvement in science – everyone's welcome! We're flooding campus with posters.

Peter: Nice! And the big **Galaxy Mapping** one?

Emma: That's the flagship! That doesn't need **special tools**, using our new telescope. (28) We offer rewards to those who demonstrate exceptional performance.

Peter: Sweet incentive! What about – **Light Pollution Survey**?

Emma: *That's* the tricky one! Teams map sky brightness across different locations. Sounds simple (29) but the environment there is extremely tough. Things like traffic noise, uneven pavements, sudden rain... Plus finding safe spots after dark? That can be nearly impossible sometimes!

Peter: Respect for managing all that! And the last one - Ice Crevasse Protocols.

Emma: The Ice crevasse protocols require long travel and constant ice probing; (30) There are many rules here because some situations can be dangerous – including no night work alone, mandatory safety gear. Can't risk accidents.

Peter: So many challenges. Perhaps you can consult our tutor to determine which research projects are most suitable.

Emma: Yeah, I intend to visit the professor right after tomorrow's classes.

That's the end of Part 3. You now have half a minute to check your answers.

Part 3.

You will hear two students discussing a course called Personal Support Workers, PSWs. First you have some time to look at questions 21 to 24.

Now listen carefully and answer questions 21 to 24.

Liam: Hey Sophie, Glad to see you here! I'm preparing for the essay for the PSWs, you know, Personal Support Workers for the elderly, and I hope you can help me with some it. Could we go over the PSW programme details again? I want to make sure I didn't miss anything from the lecture.

Sophie: Good idea. Let's start with the main duties of a PSW. **(21) The instructor said that our main job is helping with basic care stuff like bathing, getting dressed, and moving around.**

Liam: Right. I remember **the instructor also clarified that - unless the client has serious difficulties, feeding is usually taken care of by family or other trained staff.**

Sophie: Exactly. And I initially thought *chatting with our clients* was essential, but the instructor pointed out that's not our primary concern.

Liam: Yeah. And remember the other key point? "Our priority is to monitor clients' well-being. **(22) Even if they can't fully care for themselves, we ensure their environment is hazard-free and they're stable.**" For example, removing loose rugs to prevent tripping, installing grab bars in bathrooms, and ensuring proper lighting in hallways—these are all part of fall prevention strategies.

Sophie: Absolutely! administer medication? That's strictly for nurses—PSWs might only need to remind clients to take pills.

Liam: Yes! Now the entry requirements of our programme. **(23) One of the speakers mentioned he scrambled to finish high school credits last summer — they wouldn't even look at his application without the diploma.** So that's a must.

Sophie: That's reasonable. But why not *maths*?

Liam: Oh! The lecturer explained that basic numeracy for dosage is enough — no formal course required. **(24) But you absolutely needed first aid qualification — this related experience is mandatory.** And about language, fluency helps, but they assess communication skills during interviews, not through certificates.

Sophie: Right! And unless doing home visits, a clean licence isn't essential. That's a relief for me.

Liam: Yeah!

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Sophie: I didn't expect there would be so many courses to learn, in order to be a personal support worker. But we survive, eventually. And I learnt a lot from all the courses.

Liam: Absolutely! For *Understanding is Essential*, at first, I thought it was about listening, (25)

but this includes not just formal emails and meetings, but also clear, respectful interactions between colleagues, it's very helpful.

Sophie: Right! And *Foundations* was eye-opening. I just realized something I'd never noticed

before. (26) The influence of PSW in healthcare is immense. Many organizations view them as leaders in patient care, setting standards for compassionate, hands-on support.

Liam: That's so good! And for me, the most demanding course is *The Way We Work*. It's so

confusing for me at first. I expected psychology, (27) but it was all anatomy—how muscles weaken with age, the impact of arthritis on joints, even digestive changes. As the time went by, I became familiar with most of the terms.

Sophie: Good for you! And the course *Residence Care* was pretty much what the name told us, and

it's very practical. We studied case studies—like modifying kitchens for wheelchair access, installing stairlifts, and using non-slip flooring. (28) It's all about empowering clients to live alone safely so that they can enjoy a greater degree of autonomy.

Liam: Right! That's the best part of the course. And *Dynamics* got emotional. We role-played tense family meetings—like convincing a daughter her dad needs a walker. **(29) It focused on teaching us pressure management techniques - truly valuable skills and we can apply them on work and daily life.**

Sophie: Yes! Those technique will benefit us in the rest of our life. Lastly, *The Power Within*. We designed activities like puzzle sessions, reminiscence therapy with old photos, and even music playlists to trigger the client's memories. **(30) During the session, most of the clients can have a vivid recollection of their childhood, their best friends and the time they spent with their families.** The goal is to slow cognitive decline, which is one of the inevitabilities in aging process.

Liam: Yeah. That makes me sad, but there is nothing we could do.

That's the end of Part3. You now have half a minute to check your answers to part3.